

CALIMESA FIRE DEPARTMENT

POLICY MANUAL

VOLUME 6

Policy: 600.14 MCI / Scene Management

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This guideline establishes a standard structure and procedure for the operation of Fire Department units at multi-patient/mass casualty incidents. The system may be applied to any multi-patient or mass casualty incident regardless of the number of patients or incident size. This procedure shall be integrated into the overall incident management system and may include major transportation incidents, explosions or fire with multiple injuries, hazardous materials incidents with exposure victims and structural collapse incidents.

The guideline is to integrate the multi-patient/mass casualty procedures within the framework of the incident management system. It is the responsibility of the first-arriving company officer to implement these procedures on EMS incidents requiring the commitment of a two-and-one medical (2-1-M) or greater.

For the purposes of this procedure a “multi-patient incident” is defined as any incident with fewer than twenty (25) patients. A “mass casualty incident” is defined as any incident involving 25 to 100 patients. A “disaster” is defined as any incidents involving more than 100 patients.

GUIDELINE

The first-arriving company officer at the scene of a multi-patient or mass casualty incident shall establish Command. The initial Incident Commander (IC) shall remain in Command until Command is transferred or the incident is stabilized and Command is terminated. Command is responsible for the completion of the tactical objectives. The general tactical objectives listed in order of priority are:

1. Remove endangered occupants and treat the injured.
2. Stabilize the incident and provide for life safety.
3. Ensure the functions of triage, extrication, treatment and transportation are established as needed and performed appropriately.
4. Provide for the safety, accountability and welfare of rescue personnel and victims.
5. Conserve property.

In addition, the EMS TACTICAL objectives to be completed during any multi-patient/mass casualty incident include:

1. Completion of a “Triage Report”
2. Declaration of “All IMMEDIATES Transported”

The Incident Command System is used to facilitate the completion of the tactical objectives. The IC is the person who drives the Command system towards that end. The IC is responsible for building a command structure that matches the organizational needs of the incident to achieve the tactical priorities.

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When possible patients should be treated and transported in the following priority order:

1. Immediate
2. Delayed Patients upgraded to Immediate
3. Delayed
4. Minor

BASIC OPERATIONAL APPROACH

The initial actions of the first arriving officer shall be directed toward scene size-up and requesting appropriate resources and initial organization of the scene.

Initial actions include:

1. Give an on-scene report and assume command. Initiate triage.
2. Perform a rapid hazard assessment and establish a safe zone to operate.
3. Initiate traffic control and provide a safe work/treatment area.
4. Provide for occupant protection (charged hand line).
5. Call for additional resources.
6. Radio a Triage Report to Dispatch.
7. Stabilize hazards and/or remove patients to a treatment area.
8. Assign crew(s) specific task(s) to accomplish.
9. Early sectorization (triage, extrication, treatment, and transportation) or by location (north, south, east, west).
10. Initiate patient assessment and treatment functions.
11. Coordinate patient transportation.

Responding personnel are encouraged to use triage tags and IMMEDIATE labels on smaller multi-patient incidents. Triage tags should be used any time there are three (3) or more immediate patients or more than ten (10) patients. In the multi-patient incident scenario most often a multiple vehicle collision use of the START triage system can greatly improve initial scene organization and enhance its use during mass casualty incidents.

S.T.A.R.T.

This system allows first responders to triage patients in sixty (60) seconds or less based on three physical assessments: ventilation, perfusion, and mental status.

1. Deceased: No ventilation present even after attempting to position airway.
2. Immediate: Ventilation is present only after positioning the airway.
 - a. or Respirations over 30 per minute.
 - b. or Peripheral Pulse absent and Cap Refill over 2 seconds.

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- c. or Mental Status depressed, i.e., patient fails to follow simple commands.
- 3. Delayed: Any patient who does not fit the Immediate or Minor categories.
- 4. Minor: These patients are separated from the general group at the start of the triage by requesting those who can walk to go to an assigned area.

COMMUNICATIONS

- 1. All responding units will be informed of the channel and will use it for all incident radio communications.
- 2. Responding units will not contact a base hospital (BH) prior to arrival on-scene.
- 3. The assigned Medical Communications Coordinator (Med Com) shall initiate contact as soon as possible with the closest most appropriate base hospital.
 - a. Med Com's initial contact shall include:
 - i. Identifying self as Med Com.
 - ii. Name and location of the MCI Incident.
 - iii. ETA to closest hospital and trauma center to establish a point of reference.
 - iv. Scene description including any special circumstances.
 - v. Number of patients and request for MCI bed availability.
 - vi. The base hospital will use the ReddiNet to poll hospitals for bed availability. (Consider out of-county hospital(s) for polling based upon incident location.)
 - vii. Receiving hospitals will input bed availability promptly.
- 4. Med Com's second contact to the base hospital shall include:
 - a. Receiving bed availability from the base hospital
 - b. Current patient count by Patient Status (Immediate, Delayed, or Minor).
 - c. Transportation destination considerations: Pediatrics, Burn and Trauma.
 - i. The base hospital will coordinate patient destination with Med Com.
 - ii. Receiving hospitals will monitor and use the ReddiNet.

ARRIVAL

The first arriving company officer at a multiple patient incident will assume Command and give an on-scene report that will answer the question "What do I have?" "What action will I take?" "What resources do I need?" The type of situation and the approximate number and condition of patients should be communicated to Dispatch as soon as possible.

Command should rapidly survey the scene to identify any hazards or safety concerns and establish a safe zone for crews to operate. This can be accomplished through proper defensive rig positioning, use of flashing lights and the placement of flares or reflectors. Additional traffic control should be requested from law enforcement through Dispatch.

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Command should immediately request additional assistance if the need is indicated. Dispatch will begin to notify other agencies and medical facilities based on the amount of assistance requested at the scene and the progress reports from Command. The initial reports should indicate the scale of the incident to allow Dispatch to notify other agencies.

Triage will be initiated early in an incident especially when the number of patients and/or the severity of their injuries exceed the capabilities of the on-scene personnel to provide effective extrication, treatment and transportation.

Once triage is complete a Triage Report should be radioed to Alarm. A Triage Report at a two-vehicle collision may sound like: "Triage to Command. Triage is complete. We have 9 total patients: 2 IMMEDIATES, 3 DELAYED and 4 MINORS." A Triage Report signifies that triage has been completed and communicates to all responding crews the size of the major-medical incident. It also provides essential information regarding decisions to call for additional resources or to scale back the response.

The first arriving company officer needs to quickly determine the most effective means to treat patients. In incidents with few patients it may be more effective to treat patients "in place." At EMS incidents with a greater number of patients a treatment area should be established. In a case where two or more distinct groups of patients are separated by distance multiple treatment areas may be needed. Treatment area(s) can be clearly identified by using colored salvage covers (red, yellow and green) to designate treatment areas for IMMEDIATE, DELAYED or MINOR patients.

If the incident involves a building collapse or a hazardous material release, it may be more effective to remove victims to a safe area rather than stabilize hazards. This is also true of motor vehicle collisions involving a train wreck or bus. In these cases triage will be performed at the entrance to the treatment area.

STAGING

Additional resources should be requested using standard assignments and alarms as much as possible. This will facilitate an incremental approach to the incident, similar to firefighting operations, and provide predictable resources.

Command should consider implementing Level II Staging early in the incident. All First-Alarm-Medical Incidents (or greater) require a Level II Staging Area for all fire department resources, including rescue companies.

All outside agencies responding to a medical incident should be sent to the Staging Area. This

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area should be at a sufficient distance to keep the scene clear and maintain access. Staging officer will assign units as directed by Command.

Units assigned to sectors, unless carrying special equipment, should park at a distance from the scene. This parking area should be located out of the access paths. Crews should report to Extrication or Treatment Sectors carrying their medical equipment. If a treatment area is designated medical equipment and supplies should be stockpiled there.

Apparatus with extrication tools or other heavy equipment needed at the scene should be brought up closer to the actual incident site.

COMMAND RESPONSIBILITIES

The Incident Commander (IC) is responsible for the strategic level of the command structure and should:

1. Determine the appropriate strategy
2. Establish overall incident objectives
3. Set priorities
4. Develop an action plan, communicate plan.
5. Obtain and assign resources.
6. Planning—based on evaluating interventions and predicting outcomes
7. Communicate specific objective to tactical level units
8. Initiate a Unified Command with other agencies

BASIC SECTORS

Most multiple-patient incidents require patient triage, extrication, treatment, and transportation. Because of potential vehicle congestion at the site a staging sector for apparatus is also a major consideration during larger incidents.

These needs form natural basic sectors for the Incident Management System. Additional sectors may be assigned depending on the situation consistent with the Incident Management System.

The purpose of Extrication Officer is to determine in conjunction with Triage the location number and condition of patients and whether triage will be performed before or after patients are extricated from the impact area. Extrication is also responsible to assign and supervise extrication teams, extricate and deliver patients to the treatment area, and notify Command when all patients have been removed from impact area. MINOR patients who were directed earlier in the incident by triage teams to an Assembly Area will be assessed by Extrication and delivered to the treatment area if further medical care is warranted.

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The purpose of Transportation Officers is to obtain all modes of transportation needed to take patients to the hospital. Transportation should determine in conjunction with Command the location of the staging area, rescue loading area and helicopter landing zone. Transportation Sector is also responsible to determine hospital availability through the Dispatch Center, coordinate patient allocation with Treatment and supervise the movement of patients from the treatment area to the ambulance loading area or helicopter-landing zone.

ADDITIONAL SECTORS

SAFETY SECTOR

Command should assign a Safety Officer as soon as the basic sectors have been established.

As the incident escalates a Staging Officer may be required. To avoid scene congestion a Level II staging area will be identified for any First Alarm Medical incident.

If helicopters are used an LZ Sector will be established with a landing zone a safe distance from the scene. LZ Sector will keep track of patient destination, communicate landing instructions with incoming and outgoing aircraft and enforce established safety standards for landing zones (Brush Fire Air Support). At least one Engine Company will be assigned to the LZ.

BRANCHES

A mass casualty incident may require the implementation of a separate “Medical Branch” and “Transportation Branch.” Each would directly report to Command.

The Medical Branch Director is responsible to ensure that the functions of triage, extrication, and treatment are carried out. The Medical Branch Director should supervise and coordinate personnel assigned, determine and request resources needed and recommend the expansion of the command organization as needed.

Medical Branch should communicate direction and objectives to tactical units, ensure objectives are completed and maintain incident documentation.

Additional positions within the Medical Branch may include an IMMEDIATE, DELAYED and MINOR Treatment Sector Officers, Medical Communications Sector, Medical Supply Sector, Ground Ambulance Coordinator and Morgue Officer.

RESOURCE COMMITMENT AND FLOW

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Resource commitment typically follows patients. Initially Extrication will require a large resource commitment. As patients are extricated and moved to treatment, resources for extrication will decrease. These crews can be re-allocated to the treatment function.

In a disaster level incident, some fire department resources may need to be allocated to receiving hospitals until those facilities can obtain adequate hospital staff.

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